MEDSPA STANDARDS

PROTOCOL KIT

Skin & Laser Protocols

Nine protocols covering the full skin and energy-device menu — selection frameworks, mandatory test spotting, treatment endpoints, the laser safety program, and the complication pathway when something goes wrong.

9 SOPS · VERSION 1.0 · EFFECTIVE JANUARY 2026

Contents

Before you put these into service

1. Have your medical director insert the practice-specific figures: fluence and pulse ranges per device, isotretinoin and tanning exclusion windows, test patch review intervals, and peel concentrations.
2. Designate a laser safety officer in writing and complete the device inventory in SKN-008 this week.
3. Audit your eyewear against every wavelength you operate — check the OD marking is legible on every pair and that each is stored with its device.
4. Adopt the Fitzpatrick questionnaire in SKN-007 and add a fixed skin-type field to your chart template.
5. Confirm the approved topical list in SKN-003 and remove anything else from the microneedling room.

6. Print the quick reference cards, laminate them, and post them in each treatment room.

SKIN & LASER

Laser Hair Removal Protocol

Wavelength and parameter selection by skin type, test spotting, endpoints and interval planning

DOCUMENT	VERSION	EFFECTIVE	REVIEW CYCLE
SKN-001	**1.0**	**January 2026**	**Annual**
APPLIES TO	OWNER	CLASSIFICATION	SUPERSEDES
All laser operators	**Medical Director**	**Confidential**	**All prior versions**

1 Purpose & Scope

This protocol governs laser and light-based hair reduction across all device platforms. It covers device and parameter selection against skin type, the screening that prevents most burns, mandatory test spotting, treatment endpoints, and interval planning by anatomical area.

THREE FAILURES CAUSE ALMOST EVERY BURN

Parameters wrong for the skin type. Test spot skipped. A photosensitizing medication or recent tanning nobody asked about. All three are prevented by the checklist in this protocol, and all three appear repeatedly in complaint files.

MANUFACTURER IFU GOVERNS

This protocol provides the selection framework and the safety process. The specific fluence, pulse duration, spot size and cooling settings for your device come from the manufacturer's instructions for use and from your medical director. Never copy numeric settings from a template, a colleague at another clinic, or a training course on a different platform.

2 Wavelength Selection

Longer wavelengths penetrate deeper and are absorbed less by epidermal melanin, which makes them safer in darker skin. Shorter wavelengths are more strongly absorbed by melanin, which makes them more effective on fine or light hair in fair skin and more dangerous in pigmented skin.

GENERAL SUITABILITY — CONFIRM AGAINST YOUR DEVICE IFU

Wavelength	Typical platform	Best suited to	Caution
755 nm	Alexandrite	Fitzpatrick I–III with fine to medium hair. High efficacy.	High melanin absorption. Generally avoided in Fitzpatrick IV and above.
810 nm	Diode	Fitzpatrick I–IV. The most versatile general-purpose option.	Use longer pulse durations and robust cooling as skin type darkens.
1064 nm	Nd:YAG	Fitzpatrick IV–VI. The safest option in darker skin.	Less efficient on fine or light hair. More painful; plan anesthesia and cooling.
Broadband (IPL)	Intense pulsed light	Fitzpatrick I–III only.	See SKN-002. Not used for hair reduction in Fitzpatrick IV–VI in this practice.

PARAMETER DIRECTION OF TRAVEL

As skin type darkens: longer wavelength, longer pulse duration, lower fluence, more aggressive epidermal cooling, and a more conservative starting point. As hair becomes finer or lighter: efficacy falls across all platforms, and this must be discussed before the patient purchases a package. White, grey and true blonde hair does not respond.

3 Screening & Exclusions

- [ ] Fitzpatrick assessment completed and documented per SKN-007.
- [ ] Photosensitizing medication screen — including but not limited to isotretinoin, tetracyclines, St John's Wort, amiodarone, thiazides, sulfonamides and topical retinoids.
- [ ] Recent sun exposure, tanning bed use or self-tanner within the exclusion windows set by the medical director.
- [ ] Active infection, open lesion, eczema or psoriasis in the treatment area.
- [ ] History of keloid or hypertrophic scarring.
- [ ] History of herpes simplex where treating perioral or bikini areas.
- [ ] Pregnancy — practice policy set by the medical director; most defer treatment.
- [ ] Recent chemical peel, resurfacing or injectable treatment in the same area.
- [ ] Tattoos or permanent makeup in the field — these are avoided, not treated over.
- [ ] Photosensitizing conditions, connective tissue disease, or a history of photo-exacerbated disorder.
- [ ] Gold therapy — an absolute contraindication for most laser treatment.
- [ ] Realistic expectation discussion completed: reduction rather than permanent removal, multiple sessions required, maintenance likely.

ISOTRETINOIN

The traditional six-month washout is increasingly debated in the literature, but it remains the conservative standard and the position most easily defended. Your medical director must set the practice interval explicitly and record the reasoning. Do not let individual operators decide this case by case.

4 Test Spotting

MANDATORY — NO EXCEPTIONS

A test spot is required for every new patient, and again whenever the device, the wavelength, or the treatment area changes materially. A returning patient being treated on a newly acquired platform is a new patient for this purpose. 'They've had it before' is not a substitute.

1. **Select a discreet site** within, or immediately adjacent to, the intended treatment area so the skin is representative.
2. **Apply the intended treatment parameters** — a test at settings you do not plan to use tells you nothing.
3. **Deliver a small number of pulses** and photograph the site.

4. **Observe the immediate response** for several minutes. Perifollicular erythema and mild edema is the desired endpoint.
5. **Wait the interval** set by your medical director before full treatment. Delayed reactions are the ones that matter, and a longer interval is used for Fitzpatrick IV–VI.
6. **Review the site** and record the finding. Go, no-go, or adjust parameters.
7. **Document** site, parameters, immediate response, review date and outcome.

Test spot response	Interpretation
Perifollicular erythema and mild edema	Desired endpoint. Proceed.
No visible response	Under-treatment. Reassess parameters with the medical director before increasing.
Immediate greying or whitening of the epidermis	Epidermal injury. Stop. Do not treat at these settings.
Blistering, crusting or pigment change at review	Excessive thermal injury. Do not proceed. Medical director review required.
Prolonged or severe pain during the pulse	Excessive energy. Reduce and reassess.

5 Treatment Procedure

1. **Confirm consent, screening and test spot outcome** before the device is powered on.
2. **Eyewear on everyone in the room** — patient, operator and observers — at the correct optical density for the wavelength in use. See SKN-008.
3. **Prepare the area.** Shaved, clean, dry, free of makeup, deodorant, lotion and residue. Do not treat unshaved skin; surface hair causes epidermal burns and singeing.
4. **Confirm parameters against the chart and the IFU**, and confirm the skin type recorded matches the patient in front of you.
5. **Engage cooling** — contact, cryogen or air — per the device requirements.
6. **Treat methodically** in a defined pattern with appropriate overlap. Track coverage so no area is double-pulsed inadvertently.
7. **Monitor the endpoint continuously.** Stop immediately on any greying, whitening, blistering or disproportionate pain.
8. **Post-treatment cooling** and a bland emollient as directed.
9. **Written aftercare** supplied and explained: sun avoidance, no heat or friction, no active skincare, and warning signs with contact instructions.

10. **Record** parameters, passes, total pulses, areas treated, endpoint observed and any adverse response.

6 Intervals, Series & Paradoxical Hypertrichosis

Area	Typical interval	Note
Face and neck	Shorter intervals	Faster hair cycle. Higher paradoxical hypertrichosis risk.
Axilla and bikini	Intermediate intervals	Responds well; commonly the best outcomes.
Legs, arms, back and chest	Longer intervals	Slower cycle; lengthen the interval as the series progresses.

Set the specific intervals with your medical director. Extend the interval as response improves rather than treating on a fixed calendar — treating too soon wastes a session because hair is not in the growth phase.

PARADOXICAL HYPERTRICHOSIS

A minority of patients develop increased fine hair growth in or adjacent to the treated area, most often on the face and neck and more commonly in Fitzpatrick III–VI. Disclose this risk in consent before the first treatment, not after it appears. If it occurs, photograph it, notify the medical director, and reassess the treatment plan rather than simply increasing energy.

7 Quick Reference

QUICK REFERENCE CARD

Before every LHR pulse

1. **Fitzpatrick documented.** Parameters match the skin type in front of you.
2. **Meds and tanning screened** today, not at the first visit.
3. **Test spot done and reviewed** — new patient, new device, or new area.
4. **Eyewear on everyone.** Correct OD for this wavelength.
5. **Skin shaved, clean, dry.** Never treat unshaved.
6. **Cooling engaged.** Confirm before the first pulse.
7. **Endpoint = perifollicular erythema.** Greying or whitening → stop.
8. **Record parameters, passes and endpoint** every session.

Print, laminate and post at the point of care

8 Medical Director Authorization

This protocol is adopted as a standing order for the practice named below. By signing, the medical director confirms the protocol has been reviewed against current clinical guidance, applicable state scope-of-practice rules, and the training level of the staff authorized to perform it.

PRACTICE NAME

LOCATION / SITE

MEDICAL DIRECTOR — PRINTED NAME & CREDENTIALS

LICENSE NUMBER & STATE

SIGNATURE

DATE ADOPTED / NEXT REVIEW DATE

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SKIN & LASER

IPL & Photofacial Protocol

Filter selection, skin type exclusions, endpoints, and the melasma caution

DOCUMENT	VERSION	EFFECTIVE	REVIEW CYCLE
SKN-002	1.0	January 2026	Annual
APPLIES TO	OWNER	CLASSIFICATION	SUPERSEDES
All IPL operators	Medical Director	Confidential	All prior versions

1 Purpose & Scope

Intense pulsed light delivers broadband, non-coherent light filtered to a target range. That breadth makes it versatile and makes it the device most likely to cause pigmentary complications in the wrong patient. This protocol covers indication and filter selection, the exclusion criteria that are firm, endpoints, and series planning.

SKIN TYPE EXCLUSION

IPL is used in this practice for Fitzpatrick I–III only. Broadband output is absorbed strongly by epidermal melanin, and burns, hypopigmentation and post-inflammatory hyperpigmentation in Fitzpatrick IV–VI are well documented. Patients above the threshold are offered an appropriate laser alternative or referred — they are not treated on lower settings.

2 Indication & Filter Selection

FRAMEWORK — CONFIRM CUTOFFS AGAINST YOUR DEVICE IFU

Indication	Typical cutoff range	Target
Superficial pigmentation, lentigines, sun damage	Shorter cutoff	Melanin. Endpoint is darkening of the lesion.
Vascular — telangiectasia, diffuse erythema, rosacea	Mid-range cutoff	Oxyhemoglobin. Endpoint is vessel darkening or transient disappearance.
Mixed photodamage	Sequential or combined passes per IFU	Both chromophores. Requires more conservative parameters.
Hair reduction	Longer cutoff	Follicular melanin. See SKN-001; laser is generally preferred.
Acne — inflammatory	Per device indication	Porphyrins and sebaceous activity. Confirm the device is indicated for this use.

FILTERS ARE NOT INTERCHANGEABLE

Selecting a filter by habit rather than by indication and skin type is a common error. Confirm the cutoff, fluence and pulse structure against the IFU for every treatment, and record which filter was used.

3 Screening & Exclusions

- [ ] Fitzpatrick assessment per SKN-007. Fitzpatrick IV and above — do not treat with IPL.
- [ ] Recent sun exposure, tanning or self-tanner within the exclusion window. A tan is a contraindication regardless of baseline skin type.
- [ ] Photosensitizing medications and supplements screened.
- [ ] Melasma history — see the caution below.
- [ ] Active infection, open lesion or inflammatory dermatosis in the field.
- [ ] Recent peel, resurfacing, microneedling or injectable in the same area.
- [ ] History of keloid or hypertrophic scarring.
- [ ] Pregnancy — practice policy set by the medical director.
- [ ] Tattoos and permanent makeup in the field — avoided entirely.
- [ ] Undiagnosed or changing pigmented lesions — do not treat; refer per SKN-006.
- [ ] Test patch completed and reviewed per SKN-007.

> **MELASMA**
>
> Melasma is heat-sensitive and frequently worsens or rebounds after IPL, sometimes dramatically and sometimes after an initially good result. Treating melasma with IPL requires explicit medical director authorization, a documented discussion of rebound risk, and conservative parameters. Many practices exclude it entirely, which is a defensible position. Never treat melasma as though it were simple sun damage.

4 Treatment Procedure & Endpoints

1. **Confirm consent, screening, skin type and test patch outcome.**
2. **Eyewear on everyone**, appropriate to the emission range. Patient eye shields for facial treatment. See SKN-008.
3. **Cleanse thoroughly** — all makeup, sunscreen and residue removed. Skin dry.
4. **Apply coupling gel** at the thickness specified by the IFU. Insufficient gel is a direct cause of burns.
5. **Confirm filter, fluence, pulse duration and delay** against the chart and IFU.
6. **Maintain full crystal contact** with the skin. Partial contact concentrates energy at the edge and burns.
7. **Treat in a planned pattern** with the overlap specified by the IFU. Avoid stacking pulses on one site unless the IFU directs it.
8. **Watch the endpoint continuously** and stop on any warning sign.
9. **Cool the area** after treatment and apply a bland emollient.
10. **Written aftercare** and warning signs supplied with contact instructions.

11 **Record** filter, fluence, pulse settings, passes, areas, endpoint and response.

Observation	Interpretation
Mild erythema; pigmented lesions darken	Desired response. Continue.
Vessels darken or briefly disappear	Desired vascular endpoint.
Immediate greying or whitening	Epidermal injury. Stop immediately.
Blistering during or immediately after	Burn. Stop, cool, follow SKN-009.
Severe pain disproportionate to the setting	Excessive energy or poor coupling. Stop.
Rectangular imprints matching the crystal footprint	Overtreatment or poor overlap. Stop and reassess technique.

5 Aftercare & Series Planning

- Darkened pigment typically develops a coffee-ground appearance and sheds over days — warn the patient in advance or they will believe something went wrong.
- No picking, scrubbing or exfoliating the treated area until shedding is complete.
- Broad-spectrum sun protection daily, and strict sun avoidance between sessions. A patient who tans between treatments cannot be treated on schedule.
- No heat exposure — sauna, hot yoga, steam — for the period specified in aftercare.
- No active skincare such as retinoids or acids until the skin has settled.
- Series length and interval set with the medical director by indication.
- Reassess skin type, tanning status and medications before every session in a series, not only at the first.
- Maintenance planning discussed at the outset, since photodamage recurs with exposure.

6 Quick Reference

QUICK REFERENCE CARD

Before every IPL pulse

1. **Fitzpatrick I–III only.** IV and above → alternative or referral.
2. **Any tan is a contraindication.** Re-screen at every session.
3. **Melasma?** Medical director authorization and rebound discussion, or decline.
4. **Filter chosen by indication**, confirmed against the IFU.
5. **Generous coupling gel. Full crystal contact.** Both cause burns when neglected.
6. **Eyewear and eye shields** on everyone.
7. **Greying, whitening or blistering → stop.**
8. **Warn about the coffee-ground shedding** before they see it.

Print, laminate and post at the point of care

7 Medical Director Authorization

This protocol is adopted as a standing order for the practice named below. By signing, the medical director confirms the protocol has been reviewed against current clinical guidance, applicable state scope-of-practice rules, and the training level of the staff authorized to perform it.

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SIGNATURE	DATE ADOPTED / NEXT REVIEW DATE

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SKIN & LASER

Microneedling & RF Microneedling

Depth mapping, single-use cartridge policy, approved topicals and complication response

DOCUMENT	VERSION	EFFECTIVE	REVIEW CYCLE
SKN-003	**1.0**	**January 2026**	**Annual**
APPLIES TO	OWNER	CLASSIFICATION	SUPERSEDES
All microneedling operators	**Medical Director**	**Confidential**	**All prior versions**

1 Purpose & Scope

This protocol covers both mechanical microneedling and radiofrequency microneedling. The two share depth and sterility requirements; RF adds thermal injury risk and a different complication profile. It also governs what may be applied to the skin during treatment — the area where the most serious avoidable harm occurs.

> **YOU ARE CREATING THOUSANDS OF OPEN CHANNELS**
>
> Microneedling breaches the epidermal barrier deliberately and repeatedly. Anything on the skin surface goes into the dermis. Non-sterile products, cosmetic serums and preservatives that are entirely safe applied topically can cause foreign body granulomas, prolonged inflammation and permanent change when driven into the skin.

2 Screening & Exclusions

- [ ] Active acne with inflammatory or pustular lesions in the field — defer; needling spreads bacteria through the channels.
- [ ] Active infection of any kind, including warts and impetigo.
- [ ] Herpes simplex history where treating perioral areas — prophylaxis per the medical director.
- [ ] History of keloid or hypertrophic scarring.
- [ ] Isotretinoin within the interval set by the medical director.
- [ ] Anticoagulant therapy or bleeding disorder — assess and document.
- [ ] Immunosuppression, poorly controlled diabetes, or impaired healing.
- [ ] Pregnancy — practice policy set by the medical director.
- [ ] Recent laser, peel or injectable in the same area within the defined interval.
- [ ] Undiagnosed or changing pigmented lesions in the field — do not needle over them.
- [ ] Fitzpatrick documented; higher types carry greater post-inflammatory hyperpigmentation risk and warrant conservative depth.
- [ ] Metal allergy, pacemaker or implanted electronic device — critical for RF platforms.

3 Depth Selection

Depth is selected by anatomical zone and by indication, and reduced for thinner skin and for higher Fitzpatrick types. The mapping below is a framework; your medical director sets the practice ranges against your device IFU.

FRAMEWORK BY ZONE

Zone	Relative depth	Note
Periorbital and around the nose	Shallowest	Very thin skin. Use the minimum effective depth and reduce speed.
Forehead and temples	Shallow	Thin skin over bone; avoid excessive pressure.
Cheeks	Moderate	The thickest facial skin; tolerates the most depth.
Upper lip and chin	Moderate, reduced	Watch for pinpoint bleeding as an endpoint.
Neck and décolletage	Shallow	Thin skin, slower healing, higher risk of tram-tracking and prolonged erythema.
Acne scarring, body and stretch marks	Deepest, within IFU limits	Targeted rather than full-field. Requires explicit authorization.

ENDPOINT, NOT DEPTH NUMBER

The desired endpoint is uniform erythema with pinpoint bleeding. Aggressive bleeding, streaking or gouging means depth or pressure is excessive — reduce immediately. Chasing a depth setting rather than watching the skin is how tram-track scarring happens.

4 Sterility & Approved Topicals

- [ ] One sterile, single-use cartridge per patient. Never reused, never resharpened, never shared between patients under any circumstances.
- [ ] Cartridge opened in front of the patient where practical, and the packaging retained until the treatment is charted.
- [ ] Device body sleeved or barrier-protected and disinfected between patients per IFU.
- [ ] Sterile gloves and aseptic technique throughout.
- [ ] Skin prepped with an appropriate antiseptic and allowed to dry.
- [ ] Only products explicitly approved by the medical director applied during treatment.
- [ ] Used cartridges disposed of as sharps immediately.

Category	Position
Sterile hyaluronic acid preparations intended for this use	Generally acceptable. Confirm the specific product with the medical director.
Sterile saline	Acceptable as a glide medium.
Platelet-rich plasma	Acceptable where the practice has a validated PRP protocol and the operator is authorized. Autologous handling standards apply.
Cosmetic serums, vitamin C, retail skincare	**Never.** Not sterile, not formulated for intradermal delivery, and a documented cause of granuloma formation.
Products containing preservatives, fragrance or essential oils	**Never.** Preservatives are a specific granuloma risk in the dermis.
Topical anesthetic	Removed completely before needling begins. Cross-reference the dose limits in the Emergency Protocols kit (LAST).
Anything not on the approved list	Declined. Including products the patient brings in themselves.

PATIENT-SUPPLIED PRODUCTS

Patients will ask you to needle in a serum they bought. The answer is no, every time, and the reason is explained rather than negotiated. Record the request and the refusal.

5 RF Microneedling Specifics

- Insulated needles deliver energy at the tip and spare the epidermis; non-insulated needles deliver along the shaft. Know which your device uses — it changes the risk profile and the appropriate depth.
- Energy level, pulse duration and depth are set independently; confirm all three against the IFU for each pass and each zone.
- Epidermal cooling or contact cooling used as directed by the IFU.
- Absolute contraindications: pacemaker, implanted defibrillator, or other implanted electronic device. Screen explicitly and record.
- Metal implants in the treatment field require medical director assessment.
- Thermal endpoint is uniform erythema with mild edema. Immediate whitening, blistering or a burning odor means stop.
- Fat atrophy is a recognized risk with excessive energy over thin subcutaneous areas, particularly the temples and lower face — this is not reversible and warrants conservative settings.
- Downtime is longer than mechanical needling; set expectations accordingly.

6 Complications & Aftercare

Complication	Recognition	Response
Tram-track scarring	Linear marks following the pass direction, appearing over weeks.	Excessive depth, pressure or dragging. Photograph, notify the medical director, no further treatment pending review.
Granuloma	Persistent firm papules or nodules at treated sites, often weeks later.	Almost always a product driven into the dermis. Photograph, medical director notification, dermatology referral. Review what was applied.
Post-inflammatory hyperpigmentation	Darkening of treated areas, more common in Fitzpatrick III–VI.	Follow the PIH pathway in SKN-009. Strict sun protection.
Infection	Increasing pain, warmth, purulence, spreading erythema, fever.	Same-day medical assessment. Culture if discharge present. Antibiotics per prescriber.
HSV reactivation	Grouped vesicles, tingling, burning, usually perioral.	Antiviral treatment promptly. Review prophylaxis policy for future sessions.
Prolonged erythema	Redness persisting well beyond expected downtime.	Assess for excessive treatment. Bland care, sun protection, medical director review.

- Bland cleanser and emollient only until the barrier recovers; no actives.
- No makeup for the interval specified in aftercare — brushes and sponges are a contamination route into open channels.
- Strict sun protection; heat, sweat and exercise avoided for the specified period.
- No picking or exfoliating.
- Written warning signs supplied with same-day contact instructions.

7 Quick Reference

QUICK REFERENCE CARD

Microneedling non-negotiables

1. **New sterile cartridge, every patient.** No exceptions, ever.
2. **Approved sterile products only.** No retail serums. No patient-supplied products.
3. **Remove all topical anesthetic** before needling begins.
4. **Depth by zone**, reduced for thin skin and higher Fitzpatrick types.
5. **Endpoint = uniform erythema, pinpoint bleeding.** Streaking → reduce now.
6. **RF: screen for pacemakers and implanted devices.** Absolute contraindication.
7. **Active acne or infection in the field → defer.**
8. **Nodules weeks later = granuloma.** Photograph, escalate, review the product used.

Print, laminate and post at the point of care

8 Medical Director Authorization

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SKIN & LASER

Chemical Peel Protocol

Acid selection, priming, frosting endpoints, neutralization and depth-appropriate aftercare

DOCUMENT	VERSION	EFFECTIVE	REVIEW CYCLE
SKN-004	1.0	January 2026	Annual
APPLIES TO	**OWNER**	**CLASSIFICATION**	**SUPERSEDES**
All staff performing chemical peels	Medical Director	Confidential	All prior versions

1 Purpose & Scope

This protocol covers superficial and medium-depth chemical peels. It defines acid selection by indication and skin type, priming requirements, the layered application method with frosting endpoints, neutralization, and aftercare by depth.

SCOPE LIMIT

Deep phenol peels are not performed in this practice. They carry cardiac arrhythmia risk requiring cardiac monitoring, and belong in a setting equipped for it. Any request for a deep peel is referred.

DEPTH IS CUMULATIVE AND IRREVERSIBLE

You can always add another layer. You cannot remove one. Every peel is approached as though it will go deeper than intended, because the variables — skin prep, degreasing, sebum, prior treatment, skin thickness — are not fully predictable.

2 Acid Selection

FRAMEWORK — CONCENTRATIONS SET BY THE MEDICAL DIRECTOR

Agent	Class	Best suited to	Notes
Glycolic acid	AHA	Photodamage, texture, dullness, fine lines.	Requires active neutralization. Time-dependent — penetration continues until neutralized.
Lactic acid	AHA	Sensitive skin, hydration, mild pigmentation.	Gentler and larger-molecule. Good entry peel for higher Fitzpatrick types.
Salicylic acid	BHA	Acne, oily skin, comedones, follicular congestion.	Lipophilic; penetrates sebum. Self-limiting. Avoid in salicylate allergy and pregnancy.
Mandelic acid	AHA	Sensitive skin, higher Fitzpatrick types, acne with pigmentation.	Large molecule, slow penetration, low PIH risk. Often the safest choice in Fitzpatrick IV–VI.
Jessner's solution	Combination	Photodamage, acne, pigmentation.	Layer-dependent depth. Contains resorcinol — screen for sensitivity.
TCA, superficial to medium	Trichloroacetic	Photodamage, pigmentation, fine wrinkling, scarring.	Self-neutralizing. Depth driven by concentration and layers. Highest risk in this kit — requires the most experience and explicit authorization.

FITZPATRICK DRIVES SELECTION

Higher Fitzpatrick types carry substantially greater post-inflammatory hyperpigmentation risk. Favour gentler agents, lower concentrations, fewer layers and more sessions rather than one aggressive treatment. Priming matters more, not less, as skin type darkens.

3 Screening & Priming

- [ ] Fitzpatrick assessment and photography per SKN-007.
- [ ] Isotretinoin within the interval set by the medical director.
- [ ] Active infection, HSV, open lesions, dermatitis or recent waxing in the field.
- [ ] HSV prophylaxis for perioral peels per the medical director where history is present.
- [ ] Keloid or hypertrophic scarring history.
- [ ] Recent laser, resurfacing, microneedling or injectable in the same area.
- [ ] Photosensitizing medications and recent sun exposure.
- [ ] Pregnancy — salicylic acid in particular is generally avoided; practice policy set by the medical director.
- [ ] Allergy screen — aspirin and salicylate for BHA; resorcinol for Jessner's.
- [ ] Realistic expectation and downtime discussion completed and documented.
- [ ] Priming regimen completed and confirmed where required.

Priming

- Priming prepares the skin, improves uniformity of penetration and reduces PIH risk. It is not optional for medium-depth peels or for Fitzpatrick III and above.
- A typical regimen uses a topical retinoid and, where authorized, a tyrosinase inhibitor for a defined period before treatment. Your medical director specifies the agents and duration.

- Retinoids are discontinued for a defined interval immediately before the peel, since they increase penetration.
- Strict sun avoidance during the priming period.
- Confirm and document compliance before treating. A patient who did not prime is rescheduled, not treated at a lower concentration as a compromise.

4 Application & Frosting Endpoints

1. **Set up before you begin:** neutralizer, saline, cool water, fan and gauze all within reach. Never start a peel with the neutralizer in another room.
2. **Protect** the eyes, nostrils and lip vermilion, and occlude with petrolatum at canthi and nasolabial folds where pooling occurs.
3. **Cleanse and degrease thoroughly.** Inadequate degreasing produces patchy penetration and is the main cause of uneven results.
4. **Apply in a consistent sequence** across defined facial zones, starting with the most resilient area — forehead — and treating thinner or more sensitive zones last.
5. **Time from the first application**, not from when you finish applying. Note the start time out loud.
6. **Observe continuously** and assess frosting between layers, not after several.
7. **Add layers only to reach the intended endpoint**, never to fill the remaining appointment time.
8. **Neutralize** at the endpoint or the maximum time, whichever comes first.
9. **Cool** the skin and apply the post-peel occlusive specified for the depth used.

FROSTING LEVELS

Level	Appearance	Depth	Action
Level 0	Erythema only, no frost.	Very superficial.	Acceptable endpoint for a light peel or a first treatment.
Level I	Speckled, patchy white on an erythematous background.	Superficial — epidermal.	Common target. Stop here for most superficial peels.
Level II	Uniform white frost with erythema visible through it.	Papillary dermis.	Medium-depth target. Stop. Do not add layers.
Level III	Solid, dense white frost with no background erythema.	Reticular dermis.	**Too deep for this setting.** Stop immediately, neutralize, cool, notify the medical director. High scarring risk.

EMERGENCY STOP

Neutralize immediately and abandon the treatment for: level III frosting, disproportionate pain, grey or dusky discoloration, blistering during application, any solution contacting the eye, or the patient becoming distressed or unable to tolerate the procedure. Eye contact requires copious irrigation and same-day ophthalmology assessment.

5 Aftercare by Depth

Depth	Expected course	Aftercare
Superficial	Mild erythema and light flaking over several days.	Bland cleanser and emollient. Broad-spectrum sun protection. No actives until settled. No picking.
Medium	Marked erythema, swelling, then sheeting desquamation over one to two weeks.	Occlusive emollient, gentle cleansing, strict sun avoidance. Daily contact for the first few days. No makeup until re-epithelialized.

- No picking, peeling or pulling loose skin — this is the most common cause of scarring after an otherwise uneventful peel. Say it more than once.
- No heat, sauna, steam or vigorous exercise until healed.
- No retinoids, acids, scrubs or exfoliation until authorized.
- Strict sun avoidance and broad-spectrum protection for an extended period; PIH risk persists well past visible healing.
- Written aftercare with warning signs and same-day contact instructions.
- Follow-up appointment scheduled before the patient leaves for medium-depth peels.

CALL THE SAME DAY FOR

Increasing rather than settling pain, spreading erythema, purulence, fever, grouped vesicles suggesting HSV, or any area that looks grey, dusky or fails to re-epithelialize on schedule. Escalate to the medical director and see the patient rather than managing by telephone.

6 Quick Reference

QUICK REFERENCE CARD

Before and during a peel

1. **Neutralizer, saline and water in the room** before you open the acid.
2. **Priming confirmed.** Not primed → reschedule, don't improvise.
3. **Degrease properly.** Uneven prep = uneven peel.
4. **Protect eyes, nostrils, lips.** Occlude the pooling points.
5. **Time from the first application.** Say the start time out loud.
6. **Assess frosting between every layer.** Level II = stop.
7. **Level III, dusky skin, or eye contact → neutralize and escalate.**
8. **Tell them not to pick.** Then tell them again.

Print, laminate and post at the point of care

7 Medical Director Authorization

This protocol is adopted as a standing order for the practice named below. By signing, the medical director confirms the protocol has been reviewed against current clinical guidance, applicable state scope-of-practice rules, and the training level of the staff authorized to perform it.

PRACTICE NAME

LOCATION / SITE

MEDICAL DIRECTOR — PRINTED NAME & CREDENTIALS

LICENSE NUMBER & STATE

SIGNATURE

DATE ADOPTED / NEXT REVIEW DATE

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SKIN & LASER

Laser Resurfacing Protocol

Ablative and non-ablative fractional resurfacing: selection, prophylaxis, anesthesia and wound care

DOCUMENT	VERSION	EFFECTIVE	REVIEW CYCLE
SKN-005	1.0	January 2026	Annual
APPLIES TO	**OWNER**	**CLASSIFICATION**	**SUPERSEDES**
Authorized laser operators and supervising clinicians	Medical Director	Confidential	All prior versions

1 Purpose & Scope

Resurfacing is the highest-risk elective service in a skin practice. It creates a controlled wound over a large surface area, with real infection, scarring and pigmentary risk, and a recovery the patient must be prepared for before they consent. This protocol covers modality selection, prophylaxis, anesthesia planning, and day-by-day wound care.

AUTHORIZATION REQUIRED

Ablative resurfacing is performed only by operators specifically authorized for it on the competency matrix, with the supervising clinician available as defined by the medical director and your state's rules. Being signed off for laser hair removal or non-ablative treatment does not authorize ablative resurfacing.

2 Modality Selection

FRAMEWORK — CONFIRM AGAINST YOUR DEVICE IFU

Modality	Typical wavelength	Suited to	Considerations
Non-ablative fractional	1540 / 1550 nm	Texture, mild scarring, fine lines, patients who cannot take downtime.	Several sessions required. Lower risk. Safer across a wider range of skin types.
Non-ablative, superficial targeting	1927 nm	Pigmentation, actinic damage, tone.	Minimal dermal effect. Good pigment tool with modest downtime.
Ablative fractional	2940 nm (Er:YAG) / 10,600 nm (CO_2)	Deeper wrinkling, acne scarring, significant photodamage.	Substantial downtime. Higher infection, PIH and scarring risk. Er:YAG is less thermally aggressive than CO_2.
Fully ablative	CO_2 or Er:YAG, non-fractional	Severe photodamage and scarring.	Longest recovery, highest risk. Only where explicitly authorized; many practices refer these.

SKIN TYPE AND DENSITY

Post-inflammatory hyperpigmentation risk rises sharply with Fitzpatrick type and with treatment density. In Fitzpatrick III and above, reduce density before reducing depth, extend intervals, prime the skin, and plan more sessions. Fitzpatrick V–VI ablative resurfacing requires explicit medical director authorization case by case.

3 Screening, Priming & Prophylaxis

- [ ] Fitzpatrick assessment, photography and test area per SKN-007.
- [ ] Isotretinoin within the interval set by the medical director — the most commonly cited resurfacing contraindication.
- [ ] HSV history anywhere on the face — prophylaxis required regardless of treatment zone.
- [ ] Keloid or hypertrophic scarring history.
- [ ] Active infection, acne, or inflammatory dermatosis in the field.
- [ ] Immunosuppression, poorly controlled diabetes, smoking, or impaired healing.
- [ ] Prior radiation to the area, or prior surgical undermining such as a facelift within a defined interval.
- [ ] Connective tissue or photosensitive disease.
- [ ] Recent sun exposure or tanning.
- [ ] Realistic downtime discussion — with dates. Patients must not book resurfacing shortly before an event.
- [ ] Priming regimen completed and compliance confirmed.
- [ ] Post-procedure support confirmed: someone to drive them, and to help at home if needed.

Prophylaxis	Standard
Antiviral	Required for all facial resurfacing where HSV history exists, and commonly for all patients regardless of history for ablative treatment. Started before the procedure and continued through re-epithelialization. Agent, dose and duration set by the medical director.
Antibacterial	Not routine. Considered case by case for extensive ablative treatment or where risk factors exist. Prescriber decision, documented with reasoning.
Antifungal	Considered for extensive ablative treatment or where candidal infection risk is raised. Prescriber decision.

HERPETIC DISSEMINATION

HSV reactivation across freshly resurfaced skin can be severe, scarring and disseminated. Prophylaxis is started before treatment, not at the first sign of a lesion. If the patient did not start it, reschedule.

4 Anesthesia Planning

CROSS-REFERENCE LAST

Resurfacing anesthesia frequently combines a compounded topical over a large surface area with injected local — the exact stacking that causes local anesthetic systemic toxicity. Calculate and record the total dose in milligrams against the patient's weight before you begin, and follow EMG-007 in the Emergency Protocols kit. Lipid emulsion availability is a medical director decision that must be made before this service is offered.

- Record the patient's weight and calculate the maximum permitted local anesthetic dose before any anesthetic is applied.
- Log the running total, including anesthetic contained within any other product used.
- Topical anesthetic applied to the smallest necessary area, for a timed interval, with occlusion only where explicitly authorized in writing.
- Set a timer. Remove on schedule. Never leave a patient unattended with a large-area occluded topical.

- Remove all topical anesthetic completely before treatment begins.
- Nerve blocks and injected local administered only by authorized personnel.
- Monitor for early toxicity throughout — metallic taste, tinnitus, perioral numbness, agitation — and stop immediately if any appear.

5 Treatment & Immediate Post-Procedure

1. **Full laser safety setup** per SKN-008: eyewear, controlled area, signage, plume evacuation, fire precautions. Ablative resurfacing carries genuine ignition risk — no alcohol-based preps, no supplemental oxygen, wet drapes.
2. **Confirm authorization, consent, prophylaxis compliance and anesthesia totals.**
3. **Photograph** from standardized angles before treatment.
4. **Map the treatment zones** and record the parameters planned for each. Feathering at borders prevents demarcation lines.
5. **Confirm density, depth and passes** against the IFU. Density drives downtime and PIH risk more than depth does.
6. **Treat zone by zone**, watching tissue response continuously. Wipe debris per IFU between passes where directed.
7. **Stop immediately** on greying, charring, pinpoint bleeding beyond the expected endpoint, or loss of the expected tissue response.
8. **Cool the skin** and apply the post-procedure occlusive specified.
9. **Observe before discharge** and confirm the patient has their aftercare, medications, contact numbers and a driver.
10. **Record** zones, parameters, density, passes, endpoints and total anesthetic dose.

6 Wound Care & Follow-Up

Phase	Expected	Care
First 24–48 hours	Marked erythema, oedema, oozing, heat and discomfort.	Frequent gentle cleansing with the specified solution. Occlusive emollient kept in place continuously. Cool compresses. Head elevated when sleeping.
Days 2–5	Peak swelling, then crusting and sloughing.	Continue cleansing and occlusion. No picking. Antiviral continued. Daily contact from the practice.
Re-epithelialization	New pink epithelium as crusting separates.	Transition to bland emollient. Introduce sun protection as soon as tolerated. Still no actives.
Weeks 2–8	Prolonged erythema, gradually fading. Peak PIH risk window.	Strict sun avoidance. Consider pigment-directed therapy per the medical director. Reintroduce actives only when authorized.

ESCALATE THE SAME DAY

Pain that increases rather than settles. Spreading erythema beyond the treated field, purulence, fever or malaise. Grouped vesicles or punched-out erosions suggesting HSV. Any area failing to re-epithelialize on schedule, or appearing grey, dusky or ulcerated. Delayed re-epithelialization is the earliest warning sign of scarring.

- [ ] Daily contact for the first several days after ablative treatment.
- [ ] In-person review at re-epithelialization and again at the interval set by the medical director.
- [ ] Standardized photography at every review.
- [ ] Sun protection and pigment management reinforced at every contact.
- [ ] No further resurfacing until fully healed and reviewed.
- [ ] Any complication photographed, escalated and reviewed by the medical director.

7 Quick Reference

QUICK REFERENCE CARD

Resurfacing essentials

1. **Authorized operator only.** Check the competency matrix.
2. **Antiviral started before treatment.** Not started → reschedule.
3. **Calculate anesthetic in mg against weight.** Follow EMG-007.
4. **Fire risk:** no alcohol prep, no supplemental oxygen, wet drapes.
5. **Density drives PIH and downtime** more than depth. Reduce it first.
6. **Feather the borders.** Demarcation lines are permanent-looking.
7. **Daily contact for the first several days.**
8. **Delayed re-epithelialization = early scarring.** Escalate same day.

Print, laminate and post at the point of care

8 Medical Director Authorization

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SKIN & LASER

Vascular & Pigmented Lesion Treatment

Target selection, endpoints, and the lesions that must never be treated without diagnosis

DOCUMENT	VERSION	EFFECTIVE	REVIEW CYCLE
SKN-006	1.0	January 2026	Annual
APPLIES TO	**OWNER**	**CLASSIFICATION**	**SUPERSEDES**
All laser operators	Medical Director	Confidential	All prior versions

1 Purpose & Scope

This protocol covers targeted treatment of vascular and pigmented lesions. Its most important section is the first one: which lesions must be seen by a dermatologist before any device is used on them.

THE SINGLE GREATEST RISK IN THIS PROTOCOL

Treating a melanoma as though it were a lentigo destroys the lesion's diagnostic features, delays diagnosis, and can be catastrophic for the patient. Aesthetic staff are not positioned to distinguish benign pigmented lesions from malignant ones, and confidence developed over years of treating sun spots is not a substitute for a diagnosis.

2 Mandatory Pre-Treatment Assessment

Every pigmented lesion considered for treatment is individually assessed and documented. Blanket treatment of 'sun spots' across a field without lesion-by-lesion assessment is not acceptable practice.

Never treat — refer for dermatological assessment first

- Any lesion with asymmetry, irregular or notched borders, colour variation, diameter growth, or evolution in size, shape, colour or symptom.
- Any lesion the patient reports as new, changing, itching, bleeding or crusting.
- Any lesion that looks different from the patient's other lesions.
- Any raised, nodular or ulcerated pigmented lesion.
- Congenital naevi and atypical or dysplastic naevi.
- Any lesion you are uncertain about — uncertainty is itself the referral criterion.
- Any lesion in a patient with a personal or family history of melanoma, without prior dermatological clearance.
- Pigmented lesions on acral sites, nail beds or mucosa.

DOCUMENTED CLEARANCE

Where a lesion has been assessed by a dermatologist and cleared for cosmetic treatment, obtain that in writing and file it. A patient reporting that their doctor said it was fine is not clearance. Re-refer if the lesion changes subsequently.

3 Target & Device Selection

FRAMEWORK — CONFIRM AGAINST YOUR DEVICE IFU

Target	Typical approach	Endpoint
Superficial pigmented lesions — lentigines, ephelides	Shorter wavelengths strongly absorbed by melanin; Q-switched or picosecond pulses.	Immediate ash-white or grey change, or subtle darkening depending on device.
Deeper dermal pigment	Longer wavelengths with short pulse durations.	Subtle whitening. Under-treat rather than over-treat; multiple sessions expected.
Fine facial telangiectasia	532 nm or pulsed dye.	Vessel darkening, transient disappearance, or mild purpura depending on settings.
Diffuse erythema and rosacea	Pulsed dye or IPL where skin type permits.	Mild transient purpura or erythema per the chosen settings.
Larger or deeper vessels, leg veins	1064 nm Nd:YAG.	Vessel contraction or disappearance. Higher risk of thermal injury; conservative settings essential.
Cherry angiomas	Vascular-targeting wavelength, single pulses.	Immediate greying or contraction of the lesion.

TEST SPOT EVERY TIME

Lesion work is treated as a new treatment for test-spotting purposes whenever the target type, device or facial area changes. Treat one representative lesion, review at the interval set by your medical director, then proceed.

4 Procedure

1. **Confirm assessment and any dermatological clearance** is documented before powering the device.
2. **Photograph the lesions** individually at standardized angles and lighting. This is both a clinical record and your evidence of what was present beforehand.
3. **Map and count** the lesions to be treated. Agree the treatment field with the patient explicitly.
4. **Eyewear on everyone** per SKN-008. Metal eye shields for lesions near the orbital rim.
5. **Confirm parameters** against the IFU for the target and skin type.
6. **Treat lesion by lesion** with single pulses. Do not stack pulses on one lesion unless the IFU directs it and the endpoint has not been reached.
7. **Observe the endpoint** after each pulse. Stop on blistering, greying of surrounding normal skin, or pinpoint bleeding.
8. **Cool** and apply a bland emollient.
9. **Written aftercare** including the expected darkening and crusting sequence.

10. **Record** lesion locations, count, device, parameters and endpoints.

> **SET EXPECTATIONS ABOUT CRUSTING**
>
> Treated pigmented lesions typically darken, form a fine crust and shed over one to two weeks. Patients who were not warned believe the treatment made things worse and often pick, which causes scarring and PIH. Warn them in writing and show them a photograph of the expected appearance.

5 Complications & Aftercare

Complication	Response
Post-inflammatory hyperpigmentation	Follow the pathway in SKN-009. More likely in higher Fitzpatrick types and with sun exposure during healing.
Hypopigmentation	May be delayed and may be persistent. Photograph, notify the medical director, no further treatment to the area pending review.
Blistering or crusting beyond expectation	Manage per SKN-009. Photograph and escalate.
Purpura	Expected with some vascular settings. Warn in advance; resolves over one to two weeks.
Incomplete clearance	Common and expected. Plan a series; do not increase energy to force a result in one session.
Lesion recurrence	Pigmented lesions recur with sun exposure. Emphasize photo-protection and set maintenance expectations at the outset.
A treated lesion behaving unexpectedly	Stop treating it. Refer for dermatological assessment immediately.

- No picking or scrubbing crusts — they must shed on their own.
- Strict sun protection during healing and ongoing.
- No heat, sauna or vigorous exercise for the specified period.
- Bland skincare until fully healed.
- Review appointment before further sessions in a series.

6 Quick Reference

QUICK REFERENCE CARD

Before treating any lesion

1. **Assess every lesion individually.** No blanket field treatment.
2. **Asymmetry, irregular border, colour variation, change, new, itching, bleeding → refer.**
3. **Uncertain? That is the referral criterion.** Do not treat.
4. **Dermatological clearance in writing**, filed. Not verbal report.
5. **Photograph before**, standardized, every lesion.
6. **Test spot** for any new target, device or area.
7. **Single pulses. Watch the endpoint.** Do not stack to force clearance.
8. **Warn about darkening and crusting** or they will think it went wrong.

Print, laminate and post at the point of care

7 Medical Director Authorization

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MEDICAL DIRECTOR — PRINTED NAME & CREDENTIALS	LICENSE NUMBER & STATE
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SKIN & LASER

Fitzpatrick Assessment & Test Patching

The assessment that every other protocol in this kit depends on

DOCUMENT **SKN-007**	VERSION **1.0**	EFFECTIVE **January 2026**	REVIEW CYCLE **Annual**
APPLIES TO **All clinical staff**	OWNER **Medical Director**	CLASSIFICATION **Confidential**	SUPERSEDES **All prior versions**

1 Purpose & Scope

Every parameter decision in this kit is anchored to skin type. This protocol standardizes how Fitzpatrick type is assessed and recorded, how baseline photography is captured, and how test patches are performed, timed and interpreted.

ASSESS, DO NOT GUESS

Skin type is determined by how skin behaves in sunlight, not by how it looks across a consultation room. Two patients with similar apparent complexion can burn and tan very differently. Ask the questions, score them, and record the score.

2 Fitzpatrick Assessment

Administer the practice's standardized questionnaire at intake and record the resulting type in a fixed field in the chart, not in free text where it will be missed.

Type	Sun response	Practice implication
I	Always burns, never tans.	Highest efficacy across devices; lowest pigmentary risk. Watch for erythema-prone response.
II	Usually burns, tans minimally.	Generally straightforward across modalities.
III	Sometimes burns, tans uniformly.	PIH risk begins to matter. Prime for peels and resurfacing.
IV	Rarely burns, tans easily.	IPL excluded for hair reduction. Longer wavelengths, longer pulses, lower fluence.
V	Very rarely burns, tans profusely.	1064 nm for hair reduction. Conservative everything. Extended test patch intervals.
VI	Never burns, deeply pigmented.	Highest pigmentary risk. Most conservative parameters. Some services declined or referred.

Factors that modify the assessment

- Current tan — a tanned Fitzpatrick II behaves like a higher type. Treat to the current state, not the baseline.
- Ethnic background and family history of pigmentary response.
- History of PIH after acne, injury, waxing or prior treatment — a strong predictor and more useful than the score alone.
- History of melasma.

- Seasonal variation — reassess at every visit rather than relying on the intake score.

> **REASSESS AT EVERY SESSION**
>
> Skin type is recorded once; tanning status is checked every single time. A patient treated safely in February and burned in July was not a parameter failure — it was a screening failure.

3 Photography Standards

- [ ] Practice-owned device only. Never a personal phone. See OPS-005.
- [ ] Fixed lighting, fixed distance, fixed background, consistent camera settings.
- [ ] Standard angles per treatment area, replicated at every review.
- [ ] Makeup removed, hair secured, jewelry removed.
- [ ] No filters, no retouching, no beautification modes — many phones apply these by default and they must be disabled.
- [ ] Timestamped and stored in the clinical record system, not a camera roll.
- [ ] Baseline before the first treatment, then at defined intervals and at any complication.
- [ ] Clinical photography consent obtained separately from marketing consent, per OPS-002 and OPS-005.

> **PHOTOGRAPHS WORK BOTH WAYS**
>
> Standardized before images protect the patient and the practice equally. They evidence the result you achieved, and they evidence what was already present before you treated — including pigmentation or scarring the patient later attributes to you.

4 Test Patching

A test patch predicts the delayed response, which is the response that causes harm. Immediate observation alone is insufficient.

Situation	Test patch required
New patient, any energy-based device	Yes.
Existing patient, new device or platform	Yes — this is a new treatment.
Existing patient, new treatment area	Yes.
Existing patient, material parameter increase	Yes.
Fitzpatrick IV–VI, any energy-based treatment	Yes, with an extended review interval.
Recent sun exposure, tanning or self-tanner	Do not test or treat until the exclusion window has passed.
Medium-depth peel or resurfacing	A test area is used, sited discreetly.
Routine repeat session, same device, same area, stable parameters	No.

1. **Site the patch** within or immediately adjacent to the intended treatment area so the skin is representative — not on the forearm for a facial treatment.

2. **Use the intended parameters.** A conservative test tells you nothing about the treatment you plan to deliver.
3. **Photograph** the site before and immediately after.
4. **Observe the immediate response** and record it.
5. **Wait the review interval** set by your medical director — longer for Fitzpatrick IV–VI, since PIH is delayed.
6. **Review in person** and photograph again. A patient-reported review by text is not adequate for a first patch in higher skin types.
7. **Record the go / no-go decision** and any parameter adjustment made as a result.

Review finding	Decision
Expected endpoint, settled by review	Proceed at tested parameters.
Minimal or no response	Reassess parameters with the medical director before increasing energy.
Prolonged erythema	Reduce parameters and re-test.
Blistering or crusting	Do not proceed. Medical director review. Follow SKN-009.
Any pigment change, light or dark	Do not proceed. Medical director review. Reconsider modality.

5 Quick Reference

QUICK REFERENCE CARD

Every new patient, every new device

1. **Score Fitzpatrick with the questionnaire.** Record in a fixed field.
2. **Ask about PIH after acne or injury** — it predicts better than the score.
3. **Check tanning status every single session.**
4. **Baseline photos:** practice device, fixed setup, no filters.
5. **Test patch at the intended parameters**, in a representative site.
6. **Wait the full interval.** Longer for Fitzpatrick IV–VI.
7. **Review in person and photograph.**
8. **Any pigment change at review → do not proceed.**

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6 Medical Director Authorization

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SKIN & LASER

Laser Safety, Eyewear & Plume Control

Safety officer duties, controlled areas, wavelength-specific eyewear, fire risk and plume evacuation

DOCUMENT	VERSION	EFFECTIVE	REVIEW CYCLE
SKN-008	1.0	January 2026	Annual
APPLIES TO	**OWNER**	**CLASSIFICATION**	**SUPERSEDES**
All staff, including non-clinical	Medical Director	Confidential	All prior versions

1 Purpose & Scope

This is the occupational safety document for the laser program. It covers the laser safety officer role, controlled area requirements, eyewear, fire prevention, surgical plume, device custody and the records that evidence the program exists.

> **A DIFFERENT REGULATOR**
>
> Laser safety sits primarily under occupational safety authority rather than your medical board — different inspectors and different penalties, and obligations you hold as an employer regardless of clinical licensure. Most practices own the eyewear and not the program.

2 Laser Safety Officer

A named individual is designated as laser safety officer in writing, with the duties below recorded and reviewed annually. The role may be held alongside another position, but it must be assigned to a person rather than assumed by the practice generally.

- [ ] Maintain the inventory of laser and light devices with wavelengths and classifications.
- [ ] Ensure wavelength-appropriate eyewear is available, in adequate quantity, and inspected.
- [ ] Establish and maintain controlled areas and signage.
- [ ] Verify operator training and authorization before independent device use.
- [ ] Maintain device logs, service records and calibration schedules.
- [ ] Control device keys and access.
- [ ] Investigate and document every incident, near miss and unexpected outcome.
- [ ] Review the program annually and after any incident or new device acquisition.

3 Eyewear

> **EYE INJURY IS INSTANTANEOUS AND PERMANENT**
>
> Retinal damage from a laser pulse occurs faster than the blink reflex. There is no partial compliance: if the device can fire, every person in the controlled area wears correct eyewear, including the patient, observers, sales representatives and the practice owner.

Requirement	Standard
Wavelength-specific	Eyewear must be rated for the specific wavelength in use. Protection for one platform is not protection for another. Never assume interchangeability between devices.
Optical density	The OD marked on the eyewear must meet or exceed the requirement for that wavelength and power. Verify against the device IFU.
Marked and legible	Wavelength range and OD printed on the frame and readable. Retire eyewear where the marking has worn off.
Inspected	Checked before each session for scratches, cracks, pitting or delamination. Damaged eyewear is removed from service immediately.
Quantity	Enough pairs for every person who could be in the room, plus spares.
Patient protection	External goggles for body treatment. Internal metal corneal shields where treating within the orbital rim, placed by authorized personnel with topical anesthetic per protocol.
Storage	Stored with the device it protects against, not in a shared drawer where the wrong pair can be picked up.

4 Controlled Area & Fire Safety

- [ ] Treatment room designated as a controlled area whenever the device is in use.
- [ ] Warning signage at every entrance stating the wavelength and that eyewear is required. Signage displayed only while the device is in use, so it retains meaning.
- [ ] Door closed and, where fitted, interlocked. Entry procedure defined for anyone who must enter mid-treatment, including spare eyewear immediately inside the door.
- [ ] Windows covered with appropriate barrier material.
- [ ] Reflective surfaces minimized; matte or anodized instruments used in the beam path.
- [ ] Device in standby whenever it is not actively treating — never left armed and unattended.
- [ ] No alcohol-based preparations, no flammable drapes, no supplemental oxygen in the field during ablative treatment.
- [ ] Wet gauze or drapes used around the field for ablative procedures.
- [ ] Appropriate fire extinguisher accessible and in date; staff know its location.
- [ ] Hair-bearing areas near the field wetted for ablative work; singeing hair is an ignition source.

> **IGNITION RISK IS REAL**
>
> Operating room fires involving lasers are documented and devastating. The three ingredients are an ignition source, fuel, and an oxidizer. You control the fuel — alcohol preps, dry drapes, hair — and the oxidizer. Never combine supplemental oxygen with an ablative laser on the face.

5 Surgical Plume

Plume generated by ablative laser, RF and electrosurgical devices contains particulate matter, chemical byproducts and potentially viable cellular and viral material. It is an occupational exposure for staff, who encounter it repeatedly across a working week.

- [ ] Local smoke evacuation used for all plume-generating procedures, with the capture nozzle held close to the point of generation — effectiveness falls sharply with distance.
- [ ] Evacuator filters replaced on the manufacturer's schedule and the replacement logged.
- [ ] Filters handled as contaminated waste when changed, with gloves and eye protection.
- [ ] Respiratory protection appropriate to the procedure worn by everyone in the room; a standard procedure mask does not filter plume particulate.
- [ ] Room ventilation adequate; general HVAC is not a substitute for local evacuation.
- [ ] Eye protection worn — plume is an ocular irritant in addition to the laser hazard.
- [ ] Staff informed of plume risks as part of laser safety training, and the training documented.

6 Device Custody, Logs & Records

- Device keys secured and controlled by the laser safety officer, released only to authorized operators.
- Device use log recording date, operator, patient identifier, treatment area and parameters.
- Manufacturer service and calibration schedule followed, with records retained.
- Any fault, error code or unexpected output logged and the device removed from service until resolved.
- Operator authorization cross-referenced to the competency matrix in OPS-003.
- Annual laser safety training for all staff, including non-clinical staff who may enter the controlled area, with attendance recorded.
- Incident and near-miss reports retained with investigation outcomes.
- Annual program review documented, and a review triggered by any new device.

INSPECTION-READY TEST

An inspector should be able to ask for the laser safety officer by name, the device inventory, the eyewear inspection records, the training log and the device use logs — and receive all five within a few minutes. If that is not true today, that is the first corrective action.

7 Quick Reference

QUICK REFERENCE CARD

Before the device is armed

1. **Door closed. Sign up. Windows covered.**
2. **Correct-wavelength eyewear on everyone** — including observers and reps.
3. **Eyewear inspected** for scratches and legible OD marking.
4. **Patient shields** — external goggles, or corneal shields within the orbital rim.
5. **Ablative: no alcohol prep, no oxygen, wet drapes, extinguisher located.**
6. **Plume evacuation on**, nozzle close to the source, correct respiratory protection.
7. **Standby whenever not treating.** Never armed and unattended.
8. **Log the session.** Key back to the safety officer.

Print, laminate and post at the point of care

8 Medical Director Authorization

This protocol is adopted as a standing order for the practice named below. By signing, the medical director confirms the protocol has been reviewed against current clinical guidance, applicable state scope-of-practice rules, and the training level of the staff authorized to perform it.

PRACTICE NAME	LOCATION / SITE
MEDICAL DIRECTOR — PRINTED NAME & CREDENTIALS	LICENSE NUMBER & STATE
SIGNATURE	DATE ADOPTED / NEXT REVIEW DATE

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SKIN & LASER

Burn, Blister & PIH Response

Immediate cooling, depth grading, blister management and the post-inflammatory hyperpigmentation pathway

DOCUMENT **SKN-009**	VERSION **1.0**	EFFECTIVE **January 2026**	REVIEW CYCLE **Annual**
APPLIES TO **All clinical staff**	OWNER **Medical Director**	CLASSIFICATION **Confidential**	SUPERSEDES **All prior versions**

1 Purpose & Scope

Every energy-device practice will eventually cause a thermal injury. This protocol defines the immediate response, how to grade what you are looking at, when to refer, and the structured pathway for post-inflammatory hyperpigmentation — the most common long-term consequence and the one patients find hardest to accept.

> **HOW YOU RESPOND IS WHAT IS REMEMBERED**
> Patients rarely sue over a complication that was recognized immediately, explained honestly, managed attentively and followed up daily. They very often pursue one that was minimized, dismissed, or discovered by them at home with no instructions.

2 Immediate Response

1. **Stop treatment immediately.** Do not finish the area, the pass or the session.
2. **Cool the area** with cool — not ice-cold — water or cool saline-soaked gauze for a sustained period. Cooling limits the depth of the injury and reduces pain.
3. **Do not apply ice directly**, and do not apply butter, ointment, occlusive balms, toothpaste or any home remedy to a fresh burn.
4. **Assess and grade** using the table below once cooling is underway.
5. **Photograph** immediately, at standardized angles, and at every subsequent review.
6. **Tell the patient what has happened.** Plainly, without minimizing and without speculating about blame.
7. **Notify the medical director** the same day, during the visit where possible.
8. **Provide written instructions** and same-day contact details before the patient leaves.
9. **Arrange review** within 24–48 hours. Do not leave follow-up to the patient to initiate.
10. **Complete an incident report** and record the parameters and circumstances while they are fresh.

DO NOT TREAT THE REST OF THE AREA

The instinct to finish so the result is symmetrical is strong and wrong. Asymmetry can be corrected later. A second burn cannot.

3 Burn Depth Grading

Depth	Findings	Action
Superficial	Erythema only, blanches with pressure, painful, skin intact, no blistering.	Cool compress, bland emollient, sun avoidance. Photograph. Review in 48 hours. Usually heals without sequelae.
Superficial partial thickness	Blistering, moist pink base, very painful, blanches with pressure.	Do not deroof intact blisters. Non-adherent dressing. Daily review. Medical director notified same day. Infection surveillance.
Deep partial thickness	Mottled red and white, reduced sensation, sluggish or absent blanching.	Same-day referral for medical assessment. High scarring risk. Do not attempt definitive in-office management.
Full thickness	White, waxy, leathery or charred. Painless. No blanching.	Immediate referral to emergency or a burn service. Do not delay for photographs or paperwork.

REFER IMMEDIATELY REGARDLESS OF DEPTH

Burns involving the eyelid or periorbital region, the lip vermilion, the nose, the genitalia, or a large surface area. Any circumferential burn. Any burn in a patient with diabetes, immunosuppression or impaired healing. Any burn where you are uncertain of the depth.

4 Blister & Wound Management

Situation	Approach
Small intact blister	Leave intact. The blister roof is the best available biological dressing and protects against infection. Cover with a non-adherent dressing.
Large or tense blister in a mobile area	Prescriber decision. If drainage is indicated, it is performed aseptically by authorized personnel, leaving the roof in place as a dressing.
Already ruptured blister	Gentle cleansing, non-adherent dressing, infection surveillance. Do not debride in-office beyond loose necrotic material.
Crusting	Leave to separate naturally. No picking, no scrubbing, no exfoliation.
Signs of infection	Increasing rather than settling pain, spreading erythema, purulence, odour, fever or malaise. Same-day medical assessment; culture where discharge is present.

- [ ] Daily review — in person or by dated photograph — until re-epithelialized.
- [ ] Written wound care instructions supplied and explained, with a copy in the chart.
- [ ] Strict sun avoidance from the outset; PIH risk begins immediately.
- [ ] Antibiotic or antiviral cover only on prescriber decision, documented.
- [ ] No further energy-device treatment to the area until fully healed and reviewed.
- [ ] Referral to dermatology or plastic surgery for any deep injury, delayed healing or developing scar.

- [ ] Scar management plan discussed once the wound has closed.

5 Post-Inflammatory Hyperpigmentation

PIH follows inflammation of any cause and is substantially more common and more persistent in Fitzpatrick III–VI. It typically appears within weeks and can take many months to resolve. Patients need that timeline stated early and repeated, because the gap between expectation and reality is where the relationship breaks down.

1. **Photograph and document** the affected areas at standardized angles.
2. **Sun protection is the foundation** and is non-negotiable. Broad-spectrum, daily, reapplied, plus physical protection. Any ultraviolet exposure prolongs and deepens PIH, and no topical treatment will outrun it.
3. **Restore the barrier first.** Bland cleanser and emollient. Do not start actives on inflamed or broken skin — that worsens the pigmentation.
4. **Introduce pigment-directed therapy** once the skin is intact, per the medical director's authorized regimen. Tyrosinase inhibitors and retinoids are commonly used; your medical director specifies the agents, sequence and duration.
5. **Introduce one agent at a time** and reassess. Stacking actives on recently injured skin causes irritation, and irritation causes more pigment.
6. **Review at defined intervals** with photographs from the same angles.
7. **Set the timeline explicitly:** months, not weeks, and improvement is gradual.
8. **Defer any further energy-based treatment** to the area until the PIH has fully resolved and the medical director has reviewed.

DO NOT TREAT PIH WITH MORE ENERGY

The instinct to laser away the pigment you caused is a serious error. Energy delivered to recently inflamed, pigmented skin usually deepens the problem and can make it permanent. Topical management and time, under review, is the pathway.

HYPOPIGMENTATION

Loss of pigment is less common but far more difficult. It may be delayed by months and may be permanent. Photograph, notify the medical director, refer for dermatological opinion, and do not attempt to treat it with further energy.

6 Documentation & Disclosure

- Device, all parameters, passes and total energy delivered at the index treatment.
- Skin type recorded, test patch history, and screening completed at that visit.
- Time of recognition and the immediate actions taken, with times.
- Serial standardized photographs from identical angles.
- Grading applied at each review and the reasoning.

- Medical director notification time and the guidance received.
- Every patient communication, including what was said and what was provided in writing.
- Referrals made and their outcomes.
- Incident report and the root cause review — what allowed this, and what has changed.
- Manufacturer reporting where a device fault is suspected.
- Malpractice carrier notified in line with policy terms.

NEVER ALTER THE RECORD

Do not retrospectively amend parameters, screening entries or notes after a complication. Systems log it, it is generally discovered, and it converts a defensible clinical outcome into an indefensible credibility problem. Corrections are made by dated addendum only, per OPS-002.

7 Quick Reference

QUICK REFERENCE CARD

Suspected burn

1. **Stop. Do not finish the area.**
2. **Cool water or cool saline gauze**, sustained. No ice, no ointments.
3. **Grade it.** Blistering = at least superficial partial thickness.
4. **Photograph now** and at every review.
5. **Leave intact blisters alone.** The roof is the dressing.
6. **Eyelid, lip, nose, large area, or uncertain depth → refer today.**
7. **Tell the patient plainly. Notify the medical director same day.**
8. **Review in 24–48 hours.** Never leave follow-up to the patient.
9. **PIH: sun protection first. Never treat it with more energy.**

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8 Medical Director Authorization

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